

HEALTHCARE VENTURE / STRATEGY RESEARCH

Why do Chinese MedTech companies struggle to commercialize in the United States?

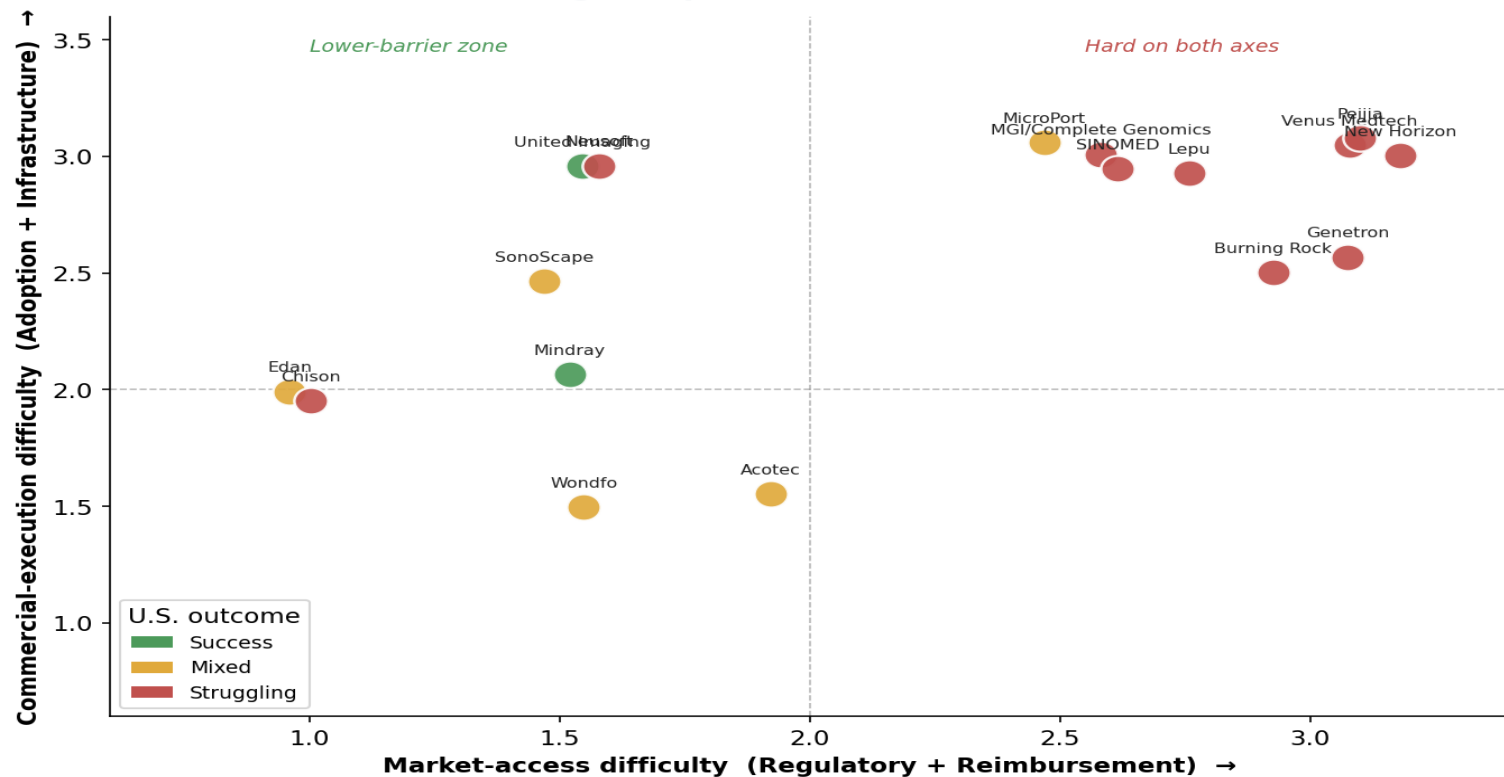
They rarely fail at the FDA. They fail downstream — at reimbursement, physician adoption, the cost of U.S. commercial infrastructure, and, increasingly, geopolitics.

Comparative case-study analysis · 20 companies · June 2026

Category — not company — predicts the U.S. outcome

Bundled-payment hardware clusters toward success; discrete-coverage implants and diagnostics cluster toward struggle.

Exhibit A — Strategic Map: Where Chinese MedTech Entrants Sit



What the map shows

Lower-left / bundled hardware

Mindray, Edan, Chison — low reimbursement friction.

Mid-map

United Imaging — moderate access, high adoption/service wall.

Top-right / hard on both

Implants (Venus, SINOMED, Lepu), diagnostics (Burning Rock, New Horizon), policy-blocked tools (MGI). Uniformly struggling.

Four barriers between FDA clearance and a paid order

FDA clearance is necessary but least differentiating. Score every entrant on these four downstream gates.

1 · Regulatory	2 · Reimbursement	3 · Physician Adoption	4 · Commercial Infrastructure
PMA/IDE trials, novel review, or IP / national-security barriers beyond a routine 510(k).	Needs its own coverage decision — CPT code, CMS NCD/LCD, payer policy, USPSTF/guideline.	Requires changing entrenched behavior, retraining, or displacing a trusted incumbent.	Large specialty salesforce, field service, clinical support, or U.S. manufacturing.
<i>'High' = a hard gate</i>	<i>'High' = a hard gate</i>	<i>'High' = a hard gate</i>	<i>'High' = a hard gate</i>

Diligence rule: if a product needs a new coverage decision AND behavior change AND a salesforce, the technology rarely matters.

Mindray

Patient monitoring · ultrasound · anesthesia · IVD

China's largest device maker (~\$5.1B group revenue, 2024). The cleanest independent U.S. success in the set.

- Strategy: acquired Datascope's monitoring business (2008) to inherit a U.S. installed base, salesforce, and service network.
- Continuous FDA 510(k) throughput 2008–2025; ultrasound #3 globally by 2024.
- Competes in categories where payment is bundled into facility reimbursement — no separate coverage decision.

#3

U.S. patient-monitoring position

~\$0.5B

Est. North America revenue (3rd-party, low confidence)

2008

Datascope acquisition = installed base

Lesson / barrier: Buy (don't build) a U.S. installed base, and win in bundled-payment categories. Neutralizes the commercial-infrastructure barrier.

United Imaging Healthcare

Diagnostic imaging • CT • MRI • PET • X-ray

Vertically integrated high-end imaging OEM (founded 2011) taking on GE, Siemens, Philips, Canon via a direct U.S. build.

- Strategy: greenfield direct entry — ~100,000 sq-ft North American HQ & plant in Houston; RSNA 2019 debut; direct sales/service.
- 16+ FDA-cleared products (uMR OMEGA, uPMR 790 PET/MR, uCT); 30,000th system shipped 2024; Seattle R&D office (2024).
- Residual wall is physician/administrator trust and service depth — not the FDA.

16+

FDA-cleared products

30,000th

System shipped from Houston, 2024

High

Adoption & infrastructure difficulty

Lesson / barrier: Doing it 'right' (U.S. manufacturing, FDA breadth, direct sales) still hits the capital-imaging trust wall. Adoption is the barrier.

MicroPort Scientific

Orthopedics • CRM • cardiovascular • robotics

Shanghai-based, HKEX-listed multi-franchise group that entered the West by acquiring incumbents.

- Strategy: buy the footprint — Wright Medical OrthoRecon (2014, ~\$290M, Arlington TN) and LivaNova CRM (2018, ~€190M).
- Imported FDA-approved portfolios and Western salesforces wholesale.
- But: carried large losses while integrating; orthopedics has lost share to Stryker, Zimmer Biomet, J&J.

~\$237M

Orthopedics revenue (2023, global)

~\$68M

1H24 adj. net loss (narrowed 63% YoY)

2

Major U.S./EU acquisitions

Lesson / barrier: M&A can buy U.S. presence but not profitable scale; integrating acquired incumbents under debt is its own failure mode.

Venus Medtech

Structural heart • TAVR

China's TAVR pioneer, pursuing the U.S. the 'right' way — FDA-grade clinical evidence — yet pre-commercial.

- Strategy: lead with IDE pivotal trials toward PMA; acquired U.S. company InterValve to broaden the system.
- First China-developed heart valve granted an FDA IDE (VenusP-Valve / PROTEUS); Venus-Vitae SMART-ALIGN spans U.S./EU/China.
- No U.S. PMA or revenue yet; faces Edwards/Medtronic/Abbott/BSX, heart-team adoption, and procedure reimbursement.

1st

China valve with an FDA IDE

\$0

U.S. revenue to date (pre-commercial)

High

On all four barriers

Lesson / barrier: The implant gauntlet: even doing everything right, IDE → PMA → coverage → heart-team adoption is years and hundreds of millions away.

Four findings that hold across all 20 companies

Exhibit B — Commercialization Difficulty Matrix

	Regulatory	Reimbursement	Physician Adoption	Commercial Infrastructure
Mindray	M	L	M	M
United Imaging	M	L	H	H
MicroPort	H	M	H	H
Acotec	M	M	M	L
Wondfo	L	M	L	M
SonoScape	M	L	M	H
Edan	L	L	M	M
MGI/Complete Genomics	H	M	H	H
Venus Medtech	H	H	H	H
Peijia	H	H	H	H
SINOMED	H	M	H	H
Lepu	H	M	H	H
Burning Rock	H	H	M	H
Genetron	H	H	M	H
New Horizon	H	H	H	H
Neusoft	M	L	H	H
Chison	L	L	M	M

Difficulty
 Low
 Medium
 High

1 • Category beats company

Every implant & molecular diagnostic scores High on reimbursement + adoption and lands in 'struggling' — regardless of clinical strength.

2 • Reimbursement & adoption bind, not FDA

Most firms obtain clearance; the graveyard is between clearance and a reimbursed, repeat order.

3 • The default 'win' is acquisition

Acotec→BSX; MicroPort is a stack of acquisitions; Mindray began with Datascope. Independent scale is rare.

4 • Geopolitics is now first-order

BIOSECURE Act / DoD 1260H foreclose genomics & diagnostics regardless of product merit (MGI).

Green flags / red flags for U.S. commercialization

Underwrite the go-to-market structure, not the technology. Run this before discounting cash flows.

GREEN FLAGS

- Bundled / existing reimbursement (no new coverage decision)
- U.S. commercial infrastructure already in place (installed base, direct sales/service)
- Low workflow disruption — slots into current physician behavior
- Strong clinical differentiation + evidence to justify switching
- U.S. manufacturing / service footprint
- Credible acquisition exit to a U.S. strategic

RED FLAGS

- FDA clearance with no reimbursement strategy
- Requires a new coverage decision (FDA→USPSTF→CMS→guideline)
- High physician switching costs (implants, robotics, EP) w/o KOLs
- Distributor-only model — no direct control or service
- Service-intensive capital equipment without U.S. service
- Country-of-origin policy exposure (BIOSECURE / 1260H / data)

KEY TAKEAWAYS

FDA clearance is the start of diligence, not the milestone.

Chinese medtech loses downstream — reimbursement, adoption, infrastructure, geopolitics.

Category beats company.

Bundled-payment hardware can win; implants and diagnostics requiring a new coverage decision rarely do.

The default 'win' is acquisition by a U.S. strategic —

underwrite to M&A, not independent scale, unless the asset is reimbursement-light and infrastructure-rich.

Strongest comparative set: Mindray · United Imaging · MicroPort · Venus Medtech (+ MGI, New Horizon, Burning Rock, SINOMED)